A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

25 May 2026

Dr Sally Wark
East Brunswixk Medical Centre

Dear Dr Wark,

Re: Ms Julie Sleight
DOB: 19/09/1959 Phone: 0419 591 710
Address: 7 Veronica Street, NORTHCOTE, VIC, 3070
Email: juliesleight1@gmail.com

Summary

- **Gastroscopy confirming healed distal oesophagitis.**
- **Chronic gastritis likely secondary to intermittent NSAID use.**
- **Arranged CT abdomen and pelvis to investigate non-specific abdominal pain.**
- **Maintenance PPI at the lowest effective dose.**
- **No action required by you.**

I had the pleasure of reviewing Julie Sleight to discuss the results of her recent gastroscopy. I am pleased to report that the moderate distal oesophagitis noted on her previous procedure has now completely healed. While there is a very small area of scarring/ring at the base of the oesophagus, it is not currently causing any symptoms of dysphagia or food bolus obstruction, so I have opted to monitor this conservatively. A small hiatus hernia remains present.

Within the stomach, I visually identified some inflammation in the antrum. Biopsies confirmed the presence of gastritis but were negative for Helicobacter pylori. Julie takes Nurofen two to three times a week for migraine management, which provides a clear explanation for these findings. Fortunately, her concurrent use of a proton pump inhibitor (PPI) appears to have protected her from more significant complications such as peptic ulceration.

We also discussed her ongoing abdominal symptoms. She describes an intermittent, non-specific pain in the central to lower right abdomen. She also reports symptoms of bloating and diarrhoea which appear to be triggered by fructose. Finally, Julie mentioned experiencing some faecal urgency or leakage, particularly when running.

Impression and Plan

I have recommended that Julie continues with her PPI at the lowest tolerated dose, perhaps trialling alternate-day therapy, to manage her hiatus hernia and provide ongoing gastric protection while she requires NSAIDs. I discussed the potential long-term side effects of PPI use, including a slightly increased risk of gastroenteritis and a potential risk of osteoporosis, but I believe the benefits of therapy outweigh these risks for her.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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I have arranged a CT of the abdomen and pelvis as a precautionary measure to investigate her abdominal pain. I suspect the results will be normal, and I have asked Julie to contact my office once the scan is complete so that I may review the imaging. Regarding her bowel habit, I have suggested she trial a fibre supplement to thicken the stool and consider seeing a pelvic floor physiotherapist to assist with her symptoms during exercise. I have not recommended a follow-up gastroscopy unless she develops new red-flag symptoms. If her functional gut symptoms remain bothersome, we may consider a trial of a low-dose neuromodulator such as nortriptyline in the future. However otherwise I have not arranged any further appointments with Julie, thank you again for referring her.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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